

MEDICAL EXAMINER NOTIFICATION AND BODY DISPOSITION

Initial Date: 10/25/2017

Revised Date: 12/27/2022

Section 8-22

Medical Examiner Notification and Body Disposition (MCA Optional Protocol)

The intent of this policy is to establish standards for proper and respectful disposition, handling, and notifications for a deceased person.

- Refer to **Dead on Scene & Termination of Resuscitation-Procedure Protocol** for determination of when and when not to initiate CPR, and when to terminate efforts.

I. Out of hospital death – Notification of the Medical Examiner

- A. The Medical Examiner's office shall be notified for any out-of-hospital death under the following circumstances:
 1. The individual dies by violence
 2. The individual's death is unexpected
 3. The individual dies without medical attendance by a physician, or the individual dies while under home hospice care without medical attendance by a physician or registered nurse, during the 48 hours immediately preceding the time of death, unless the attending physician, if any, is able to determine accurately the time of death.
 4. If the individual dies as a result of an abortion, whether self-induced or otherwise.
 5. Death of a prisoner in a jail or prison.
- B. Responsibility to notify the Medical Examiner
 1. If a patient is transported to a hospital from the scene, having met the above criteria, EMS shall notify the hospital of the criteria which requires notification. Responsibility for the notification of the Medical Examiner resides with the hospital.
 2. If a patient meeting the above criteria is pronounced dead without being transported to the hospital, the responsibility for notification of the Medical Examiner is shared between law enforcement and EMS personnel having authority for the management of the patient.
 3. Patients who do not meet the above criteria and who are pronounced dead outside of a hospital do not require notification of the medical examiner.
 - a) Any patient who is attended by a physician or registered nurse at the time of death (nursing home)
 - b) Any patient who was under home hospice care and had medical attendance by a physician or registered nurse within the 48 hours immediately preceding the time of death (hospice patient either at home or in hospice facility)

II. Out of Hospital Death – Management, Handling and Movement of Body

- A. A body shall not be moved from the location of death if any mandatory Medical Examiner reporting criteria are present, **unless the ME's office provides**

MCA Name:

MCA Board Approval Date:

MCA Implementation Date:

MDHHS Approval: 12/27/22

MDHHS Reviewed 2023

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official notification that an autopsy or external examination will not be performed and that the body will be released to the funeral home.

- B. Alternately, the body of a person who has unexpectedly died in a public location may be moved by EMS only after approval from the ME's office. Such approval shall not be requested if there is any indication of violence, criminal activity or if the physical environment may contain evidence related to a cause of death or an injury pattern.
- C. **A situation which does not require notification of the ME's office does allow for movement of the body pending retrieval by the funeral home.**
- D. Bodies must remain attended in the case of an unexpected death. Police should take custody of the body in the instance of an ME case. If there is a significant delay of the funeral home, the body may be left with the family.
- E. Medical devices utilized during care by EMS may be removed from the patient if the body is released by the ME's office to the funeral home (IV's, advanced airways, defibrillation pads, etc.)
- F. Medical devices utilized during care by EMS must remain in place if the ME's office advises that an autopsy or examination will be performed.
- G. If there is evidence of suspicious, violent, or unusual cause of death, caution should be taken to avoid contamination of the scene.
 - 1. In the instance of a scene resuscitation and termination, the identification may be removed from the body. No other personal items may be removed.
 - 2. Bodies may be covered with a sheet when the body is visible to the public or bystanders.
- H. If a body is moved, as permitted in the prior criteria, the location should be to a private, secure and nearby location pending retrieval by the funeral home or the ME's staff.
- I. Bodies must be handled with care and respect for the deceased, the family and the public.

III. Death in an Ambulance – termination of care

- A. Patients with valid DNR orders being transported for any reason, whether due to an emergency condition or during an interfacility transfer, who experience cardiac or respiratory arrest shall have the DNR honored unless, before arresting, the patient expressly withdraws their DNR.
- B. Patients for whom transport was initiated but who, during transport, meet the criteria for either Dead on Scene or Termination of Resuscitation protocols, and for whom On-line Medical Control (OLMC) has approved a termination of resuscitation (as required by those protocols respectively), may have care terminated while still en route to the hospital.

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IV. Death in an Ambulance – transportation of body

- A. In the event of a patient death in an ambulance, the body shall be transported to the original destination hospital if the call was originally from a scene to a hospital or from a facility to a hospital (transfer).
 - 1. The patient's body shall be brought to the Emergency Department
 - 2. The patient will be registered to accommodate both the transfer of custody and for preservation of evidence, if indicated
 - 3. The Medical Examiner shall be contacted by the hospital and the disposition of the body shall be according to the direction of the ME.
- B. If a patient is being transferred to a nursing home or to their home, immediately following discharge from a hospital, and death is determined, the body should be brought back to the hospital from which they were discharged, unless the patient is a hospice patient.
 - 1. If the patient is a hospice patient and hospice will be meeting you at the destination, or the destination is a hospice facility, you may continue on to the destination and relinquish the body to hospice personnel. This is permitted, without notification of the Medical Examiner, since the patient was both a hospice patient and received medical attendance within the 48 hours immediately preceding the time of death. However, if the death was unexpected, the Medical Examiner must be notified.
 - 2. If the patient is a hospice patient, and hospice personnel will not be meeting you at the destination, continue on toward the destination, contact a supervisor from your agency and evaluate the situation. Where you ultimately go is dependent on how far you are from the destination, if family was intending to meet you at the destination, if the death was unexpected and any confounding factors. The body may not be left without there being a custodial transfer from EMS to an appropriate healthcare provider.
 - a) Consider contacting the hospice care provider
 - b) Consider consultation with online medical control
 - c) If the death was unexpected, contact the Medical Examiner
- C. If a patient is being transferred from a facility to an appointment, or vice versa, where neither the starting or ending destination was a hospital:
 - a) If no DNR exists, treat and transport the patient to a hospital
 - b) If a DNR exists but the patient is not a hospice patient, determine death, honor the DNR, and transport the body to a hospital
 - c) If a DNR exists and the patient is a hospice patient, determine death; honor the DNR, refer to IV(B)(1) and (2) above.